

--SUMMARY--

Decision No. 84/19

08-Feb-2019

C.Huras

- Experience rating (NEER) (retroactive adjustment)
- Second Injury and Enhancement Fund {SIEF} (severity of accident)
- Second Injury and Enhancement Fund {SIEF} (severity of preexisting condition)

No Summary Available

13 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w1419n]

Style of Cause:

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 84/19

BEFORE:

C. Huras: Vice-Chair

HEARING:

January 8, 2019 at Toronto
Written

DATE OF DECISION:

February 8, 2019

NEUTRAL CITATION:

2019 ONWSIAT 384

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated September 20, 2017

APPEARANCES:

For the worker:

Self-represented

For the employer:

M. Senicar, Paralegal

Interpreter:

Not applicable

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
Toronto ON M5G 2P2

**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

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REASONS

(i) Introduction

[1] The employer appeals a decision of the Appeals Resolution Officer (ARO) dated September 20, 2017 which confirmed the decision granting 50% Second Injury and Enhancement Fund (SIEF) relief, in accordance with a finding of a moderate accident and a moderate pre-existing condition.

[2] The Tribunal determined that this appeal was suitable for a written hearing and the employer's representative provided a written submission on March 28, 2018. In a letter dated April 25, 2018, the Tribunal acknowledged the worker's request for an extension to provide his written submissions. Final submissions from both parties were expected by May 21, 2018. The letter stated that if submissions were not received by May 21, 2018, the Tribunal would assume that the parties had nothing further to add and the appeal would be forwarded to a Vice-Chair for a decision. In a letter dated May 22, 2018, the Tribunal confirmed that no further submissions were received and, as such, the file was released to the Scheduling department.

(ii) Issues

[3] The issues under appeal are as follows:

1. Whether the employer is entitled to an increase in SIEF relief with respect to the costs of this worker's claim.
2. If successful, the employer requests entitlement to a retroactive NEER adjustment.

(iii) Background

[4] The following are the basic facts.

[5] On September 4, 2013, this then 19-year-old barista tripped on a mat at work, sustaining injuries to his back and left knee. The worker had previously injured his back at work two weeks earlier and had a previous injury to his left knee in March 2013 from a skateboarding accident which had reportedly resolved. Entitlement was granted for a left ACL and meniscal tear. There was no entitlement for the low back.

[6] On October 4, 2013, the worker resigned from his employment with the accident employer and secured a new job with a new employer.

[7] The worker required surgery for his compensable left knee injury in March 2014. On March 24, 2016, the worker claimed a recurrence of his left knee injury, which was granted for the diagnosis of MCL strain. The worker underwent revision surgery on his left knee in July 2016.

[8] In December 2016, the employer requested cost relief under the SIEF policy, which was denied by the Case Manager (CM) in a decision dated January 23, 2017.

[9] The worker required a third compensable left knee surgery on January 24, 2017.

[10] On April 19, 2017, the CM reconsidered the previous decision of January 23, 2017 which denied SIEF relief and granted SIEF relief at 50% based on a finding that there was a moderate accident and a moderate pre-existing condition. The CM reconsidered this decision on

June 22, 2017, and upheld the previous decision. The employer objected to the CM's decisions of April 19, 2017 and June 22, 2017, which were upheld by the ARO in a decision dated September 20, 2017. The employer now appeals this ARO decision to the Tribunal.

(iv) Law and policy

[11] Since the worker was injured in 2013, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[12] The Board's authority to establish the Second Injury and Enhancement Fund derives from section 98 of the WSIA, which states:

98 (1) The Board may establish a special reserve fund to meet losses that may arise from a disaster or other circumstance that, in the opinion of the Board, would unfairly burden the employers in any class.

[13] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[14] Pursuant to section 126 of the WSIA, the Board stated that Policy Packages #247 and #300, Revision #9, would apply to the subject matter of this appeal.

[15] I have considered these policies as necessary in deciding the issues in this appeal, including *Operational Policy Manual* (OPM) Document No. 14-05-03, entitled "Second Injury and Enhancement Fund." This policy provides in part:

Policy

If a prior disability caused or contributed to the compensable accident, or if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing condition, all or part of the compensation and health care costs may be transferred from the accident employer in Schedule 1 to the SIEF.

Both physical and psychological disabilities are included.

Guidelines

There is no provision in the Act for the Fund to apply to Schedule II employers.

In situations where alcoholism plays a role in the causation of an accident, it is not considered to be a pre-existing condition with regard to the application of SIEF relief.

The objectives of this policy are to provide employers with financial relief when a pre-existing condition enhances or prolongs a work-related disability. It thereby encourages employers to hire workers with disabilities.

Definitions

Pre-accident disability is defined as a condition which has produced periods of disability in the past requiring treatment and disrupting employment.

Pre-existing condition is defined as an underlying or asymptomatic condition which only becomes manifest post-accident.

(...)

SIEF-application to employer costs

Medical significance of pre-existing condition*	Severity of accident**	Percentage of cost transfer***
Minor	Minor Moderate Major	50% 25% 0%
Moderate	Minor Moderate Major	75% 50% 25%
Major	Minor Moderate Major	90%-100% 75% 50%

NOTES

* The medical significance of a condition is assessed in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person. An associated pre-accident disability may not exist.

With psychological conditions, the possibility of prior psychic trauma resulting from life experience could be considered as evidence of vulnerability, and justify recommending relief to the employer, even in the absence of pre-existing psychological impairment.

** The severity of the accident is evaluated in terms of the accident history and approved definitions.

Accident History Components

- mechanics (lift, push, pull, fall, blow, etc.)
- position (kneeling, standing, sitting, squatting, bending, etc.)
- environment (lighting, temperature, weather conditions, terrain, etc.)

Definition – “Severity of Accident”

Minor: expected to cause non-disabling or minor disabling injury

Moderate: expected to cause disabling injury

Major: expected to cause serious disability probable permanent disability

*** The percentage of the total cost of the claim transferred to the SIEF.

[16]

In analyzing a SIEF appeal, *Decision No. 1404/11* states:

The standard of proof in a Tribunal appeal is the balance of probabilities. In a SIEF appeal, this means that the evidence must demonstrate that it is more likely than not that the underlying condition prolonged or enhanced the worker’s disability resulting from the workplace accident. The severity of the accident and the significance of the pre-existing condition must also be determined based upon direct evidence or reasonable inferences which may be drawn from the evidence. In a SIEF appeal, there is often a lack of direct evidence, and the parties may be drawn into relying upon unsupported assertions and arguments. I find it necessary to emphasize, however, that entitlement for SIEF relief must be demonstrated on the basis of valid evidence or reasonable inferences drawn from the evidence.

...

(v) Submissions

[17] The employer is seeking an increase in the quantum of SIEF, which is currently at 50%, to 90%, on the basis that the pre-existing condition was major and the accident history was minor. The worker did not provide any submissions for this employer appeal.

(vi) Analysis

[18] The appeal is allowed for the reasons set out below.

(a) Quantum of the Second Injury and Enhancement Fund cost relief

[19] As can be seen from the table of relief found in the Board's SIEF OPM Document No. 14-05-03, the amount of SIEF relief granted is dependent on two variables: the severity of the accident and the medical significance of the pre-existing condition. I will first consider the medical significance of the pre-existing condition.

(b) Medical significance of the pre-existing condition

[20] In this case, the Board determined based on a medical opinion from Dr. S. Wenzell, Medical Consultant (MC), independent practice, on April 18, 2017 that a moderate pre-existing condition likely enhanced this claim and prolonged the recovery period from the left knee injury on September 4, 2013. The employer's representative argued in his submission of March 28, 2018 that the pre-existing condition should be considered major based on the argument presented in their previous submission of March 12, 2017, and that considerable weight should be given to Dr. H. Weinberg's, orthopaedic surgeon, report of March 11, 2017.

[21] While I have considered all of the evidence before me in this appeal, in particular, I note the following evidence which relates to the worker's pre-existing condition and his disability following the work-related injury on September 4, 2013:

- A clinical note (CN) dated March 27, 2013 from the emergency department, indicated that:

S was skateboarding and fell off of board and landed on feet but when landed knee snapped laterally and then back into position, unable to weight bear at that time of the accident and today, swollen, and unable to move.

The CN also noted that the worker had pain along the joint line. He was advised that if there was no fracture, he would require physiotherapy. He was provided with a brace and crutches, instructed to take Advil, Tylenol and Tylenol 1, was authorized off work for the remainder of the week, and a referral to a sports medicine clinic was recommended. X-rays of the left knee taken on March 27, 2013 did not identify any significant abnormality.

- The worker attended the emergency department on April 12, 2013 to follow-up on two issues. At that time he reported that he "continues to have left knee pain and swelling after fall from skateboard, see note, x-ray normal, confirms was a twisting injury." At that time the physician noted:

normal gait
left knee – no [sic] red, but obviously swollen, appears to involve joint space, limited passive ROM due to pain, active ROM
– not able to proceed [sic] due to pain
diffusely tender at joint in all areas.

The physician noted that “given the mechanism of injury, and exam it seems likely there is a ligamentous injury, discussed with patient, will refer to sports medicine.” The worker was provided with a prescription for physical therapy and instructed to rest, ice, compress and elevate (RICE) the left knee.

- In a Board memorandum dated October 9, 2013, the worker advised the Eligibility Adjudicator (EA) that his left knee ligament injury in February 2013 required crutches and ambulation for one week, did not require active treatment or accommodation and he fully recovered with no ongoing symptoms for the left knee at the time of his work-related injury on September 4, 2013. I note that the worker’s recollection of his pre-existing disability, as communicated to the Board on October 9, 2013 and as recorded in this memorandum, is inconsistent with the contemporaneous evidence in the claim file which indicates that the worker continued to have significant symptoms in his left knee on April 12, 2013, which is more than two weeks after his skateboarding accident.
- An MRI taken on October 11, 2013 identified a medial meniscal tear, complete ACL tear, and focal patellar chondropathy.
- The worker had three surgeries on his left knee. The first surgery was on March 4, 2014 for an arthroscopic debridement, partial left lateral meniscectomy, left medial meniscal repair, notchplasty and ACL allograft of the left knee. The second surgery was on July 23, 2016 for a failed autograft ACL, a meniscal tear medial side (new injury from recurrence) and to treat persistently wide tunnels on the tibial side. The third surgery was on January 24, 2017 for a failed ACL reconstruction, a small radial tear of the lateral meniscus (new injury from recurrence) and to treat scar tissue on the anterior tibial plateau.

[22] In addition to the medical documentation relating to the pre-existing condition and the surgeries performed after the work-related injury on September 4, 2013, there are a number of medical opinions with respect to the effect of the pre-existing left knee condition/disability on the work-related injury.

[23] In October 2013, the CM requested a medical opinion on whether the MRI findings were compatible with the work-related injury on September 4, 2013. In his response of October 31, 2013, Dr. G. Magor, MC, independent practice, stated in part that:

...The knee popping, immediate inability to weight bear, persisting joint swelling, joint line tenderness, and inability to be fully mobile 6 weeks after the skateboarding incident point to significant intra-articular injury at that time, in the mind of this reviewer. There are no medical data for the period between May 7 and Sep 5/13 and it is not possible for me to conclude if and how the worker continued to be impaired to any degree at the time of the subject accident. However, the subject injury which was itself a significant insult clearly caused immediate and persisting impairment. **I feel that it is likelier than not that there were significant intra-articular lesions prior to the subject injury (at least partial ACL tear with likely medial meniscal tear related to the skateboarding injury) and that these were aggravated by the accident of Sep 4/13. Instability resulting from the earlier likely ACL tear may also have contributed to the extent of further damage sustained at the time of the subject injury.** [emphasis added]

[24] In March 2017, the employer’s representative obtained an independent file review from Dr. Weinberg, orthopaedic surgeon. In his report of March 11, 2017, Dr. Weinberg, stated in part that:

The MRI indicated a vertical oblique tear in the medial meniscus, irregularity in the lateral meniscus, a deep fissure in the patella and a full thickness tear in the ACL. **The fissure in the patella would indicate some pre-existing degenerative change or chondropathy. The meniscal tears and the ACL tear could have been a direct result of the work injury but this would be hard to explain on the basis of a simple trip and fall. The likelihood is that there was pre-existing pathology related to the skateboard injury which was aggravated as a result of the work injury.** [emphasis added]

...

From an orthopaedic perspective, the Worker had significant pre-existing pathology in the left knee as a result of the skateboard injury which occurred several months prior to the work injury. Following the skateboard injury, the Worker had immediate pain and swelling as well as stiffness and difficulty with weight-bearing. This would suggest significant trauma. The knee did eventually settle down apparently but the meniscal and ligamentous damage rendered the knee more vulnerable to subsequent injury. I would agree with Dr. Magor that, based on the available information there was pre-existing pathology from the skateboard injury that was further aggravated by the work injury. **Had there not been significant pre-existing pathology, it is unlikely that the work injury would have resulted in extensive damage requiring surgical intervention. From an orthopaedic standpoint, the pre-existing pathology related to the skateboard injury would be a major factor in this case.**

...

- [25] In April 2017, the CM requested a medical opinion on whether or not there was evidence of a pre-existing or underlying condition that enhanced or prolonged the recovery period. The CM also requested that the MC comment on the severity of the accident. In his response of April 18, 2017, Dr. S. Wentzell stated in part that:

Considering the above medical factors, and while respectfully noting the opinions provided by the previous medical consultants and Dr. Weinberg, a moderate pre-existing condition is evident which has likely enhanced this claim and prolonged recovery related to the left knee injury on 04Sep2013. This takes into account the worker's previous left knee skateboard injury, with likely partial ACL tear and meniscal injury, but following which the worker described symptomatic recovery and no recurrent left knee instability until after the work injury on 04Sep2013. The details provided in the medical reports support that the worker likely had a left knee partial ACL tear which progressed to a complete tear as a result of the work-related injury on 04Sep2013. [emphasis added]

- [26] With respect to the matter of the pre-existing condition, Board OPM Document No. 14-05-03 requires that it be characterized as minor, moderate or major. The policy does not define these terms and indicates only that the medical significance of a condition is assessed "in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person." These provisions were interpreted as follows in *Decision No. 1582/07*:

I interpret the policy to mean that the medical significance of a pre-existing condition should be considered to be "minor" if it made the worker slightly more liable to develop a disability of greater severity than a normal person, and that it should be considered "major" if it made the worker extremely liable to develop a disability of greater severity than a normal person. If the extent to which the pre-existing condition made the worker more liable to develop a disability of greater severity than a normal person was more than slight, but less than extreme, the medical significance of the pre-existing condition could be considered moderate.

- [27] I acknowledge the varying opinions with respect to the severity of the pre-existing condition with Dr. Weinberg classifying the pre-existing condition as major and Dr. Wentzell classifying the pre-existing condition as moderate. However, after considering the Tribunal's interpretation of a minor, moderate and major pre-existing condition, I find on a balance of probabilities that the severity of the pre-existing condition, in this case, is best classified as "major" in that it made the worker "extremely liable to develop a disability of greater severity than a normal person." In reaching this conclusion, I have considered the significance of the pre-existing condition and its effect on the worker's condition and on the work-related injury of September 4, 2013 and its sequelae; and the worker's age at the time of the accident.
- [28] There is no dispute that the worker's pre-existing left knee condition/disability enhanced the work-related left knee injury on September 4, 2013 and prolonged the recovery period. This is consistent with Dr. Magor's opinion on October 31, 2013 that:
- ...it is likelier than not that there were significant intra-articular lesions prior to the subject injury (at least partial ACL tear with likely medial meniscal tear related to the skateboarding injury) and that these were aggravated by the accident of Sep 4/13...
- ...
- [29] It is also consistent with Dr. Weinberg's opinion on March 11, 2017 that "the likelihood is that there was pre-existing pathology related to the skateboard injury which was aggravated as a result of the work injury" and Dr. Wentzell's opinion on April 18, 2017 that the worker's previous left knee skateboard accident "likely" resulted in a partial ACL tear and meniscal injury which enhanced the compensable injury and prolonged the recovery period.
- [30] However, I also find on a balance of probabilities that the worker's prior left knee condition/disability contributed to the injury on September 4, 2013 to the extent that it made the worker extremely liable to develop a disability of greater severity from a tripping accident than a normal person without any underlying pathology. This is consistent with Dr. Magor's opinion on October 31, 2013 that "Instability resulting from the earlier likely ACL tear may also have contributed to the extent of further damage sustained at the time of the subject injury," and Dr. Weinberg's opinion on March 11, 2017 that "the meniscal tears and the ACL tear could have been a direct result of the work injury but this would be hard to explain on the basis of a simple trip and fall" and his conclusion that "Had there not been significant pre-existing pathology, it is unlikely that the work injury would have resulted in extensive damage requiring surgical intervention." I accept the opinions of Dr. Magor and Dr. Weinstein that the worker's pre-existing left knee condition/disability contributed in a major way to the extent of the damage caused by the tripping accident on September 4, 2013. I accept their opinions as they were based on a review of the complete medical records. I also note that Dr. Wentzell concurred with the opinions of Dr. Magor and Dr. Weinstein in her report of April 18, 2017. I find that the pre-existing underlying pathology and instability in the left knee likely made the worker extremely liable to develop a disability of greater severity than a normal person which is consistent with the opinions of Dr. Magor and Dr. Weinstein.
- [31] Finally, I note that at the time of the work-related injury, the worker was only 19 years of age. I find on a balance of probabilities that the worker was extremely liable to develop a disability of greater severity than a normal person of the same age following a tripping accident as a result of his pre-existing left knee condition.

[32] For the reasons stated above, I find that the pre-existing condition contributed to the accident on September 4, 2013, enhanced the compensable injury and prolonged the recovery period. I also find that the worker's pre-existing condition at the time of the work-related injury on September 4, 2013, made him extremely liable to develop a disability of greater severity than a normal person. Accordingly, I find that the medical significance of the pre-existing condition is major.

[33] I will now consider the severity of the accident.

(c) Severity of the accident

[34] Board OPM Document No. 14-05-03 requires that accidents be characterized as either "minor" (expected to cause non-disabling or minor disabling injury), "moderate" (expected to cause disabling injury) or "major" (expected to cause serious disability, probable permanent disability). In addition to these definitions, the policy suggests that the severity of the accident is to be evaluated in terms of the "accident history components," which include "mechanics" (lift, push, pull, fall, blow, etc.), "position" (kneeling, standing, sitting, squatting, bending, etc.) and "environment" (lighting, temperature, weather conditions, terrain, etc.).

[35] I find on a balance of probabilities that the severity of the accident was minor "expected to cause non-disabling or minor disabling injury." In reaching this conclusion, I have placed considerable weight on the worker's reporting of the accident history.

[36] The Worker's Report of Injury/Disease (Form 6) which was completed by the worker on September 16, 2013 stated that "mats were overlapping and shoe got caught and tripped me."

[37] In a Board memorandum dated October 9, 2013 the worker provided the following accident history to the Eligibility Adjudicator (EA):

-ON 04Sep13 worker was walking to the drink machine to serve a customer when he tripped on 2floor mats overlapping and injured his left knee

-States that his left right got stuck in between 2 overlapping mats and to avoid from falling he lunged forward and his weight landed on his left foot and his knee bent outward and felt immediate pain with difficulty walking

[38] While I acknowledge that the worker reported that he "lunged forward" and that his knee "bent outward," I note that there is no evidence of significance before me to indicate that any external factors such as a push or a fall contributed to the mechanics of the accident or that positional (noting that the worker was simply walking at the time of the accident) or environmental factors contributed to the accident. Accordingly, in the absence of any other factors, I find that the severity of the accident is best classified as minor, "expected to cause non-disabling or minor disabling injury." This is also consistent with Dr. Weinberg's opinion on March 11, 2017 that the accident on September 4, 2013 was "a simple trip and fall."

[39] Tribunal jurisprudence has also determined that trip and fall accidents which do not involve any other significant mechanical, positional or environmental factors are best classified as minor accidents for SIEF purposes. In his submission of March 28, 2018, the employer's representative referred to a number of Tribunal decisions in support of his appeal, specifically, *Decision Nos. 813/05, 909/07, 615/08, 664/09, 1320/15 and 3843/17* which all involved slip and/or fall injuries. I agree with the employer's representative that the facts and circumstances before me in this appeal are similar to the facts and circumstances in *Decision No. 1320/15*,

which involved a slip and fall on ice for a worker with a background of a pre-existing condition and disability. In that decision, the Vice Chair found:

[25] In the case before me, the worker slipped on ice in a parking lot, falling on her outstretched right arm. The file indicates she was walking around her truck when she fell. She therefore was not moving quickly, and did not fall with a great deal of force or momentum behind her. I find that, in a person without the worker's pre-existing condition and disability, such a fall would be expected to cause, at the most, minor disability. The accident therefore is minor in severity.

...

[40] I agree with the analysis of the Vice-Chair in *Decision No. 1320/15* and I find on a balance of probabilities that in a person without the worker's pre-existing left knee condition/disability, a tripping accident, would be expected to cause, at the most, a non-disabling or minor disabling injury.

[41] For the reasons stated above, I find that the severity of the accident is minor.

[42] Board OPM Document No. 14-05-03 provides that a major pre-existing condition and a minor accident results in the application of 90% SIEF relief to employer costs. Accordingly, the employer is entitled to an increase in SIEF which is currently at 50% to 90%.

(d) Entitlement to a retroactive NEER adjustment

[43] The employer requested in the Notice of Appeal for Employer's dated October 31, 2017 that the Tribunal direct the Board to make a retroactive adjustment to the employer's New Experimental Experience Rating (NEER) record if an increase in the SIEF cost relief was allowed. *Decision No. 708/07* established that the Tribunal has jurisdiction to consider a request for a retroactive adjustment of the employer's NEER account in a SIEF appeal.

[44] Board OPM Document No. 13-02-07, published July 15, 2011, which was not included in the case materials but is available on the Board's website and in the Tribunal's library states that it applies "to all adjustments to NEER refunds and surcharges decisions made on or after July 15, 2011 with respect to claims with injury dates of January 1, 2008 and later." The current version of the policy states that the Board's Experience Rating (ER) section calculates the final NEER refund or surcharge for a given accident year at the time of the final review, which is now September 30 of the **fourth** year following the accident year. It also provides an additional one-year window in the event of certain Board errors.

[45] Section 126 of the WSIA requires this Tribunal to apply Board policy that is applicable to the subject matter of an appeal. Section 124(1) of the WSIA provides that the Tribunal shall make its decisions based upon the merits and justice of a case and is not bound by legal precedent. In determining an appeal on the merits and justice, Tribunal decisions have adopted the approach articulated in *Decision No. 1085/98*. That decision stands for the proposition that to grant a retroactive adjustment in circumstances that do not meet the applicable policy criteria, "the facts must be sufficiently unique that they can be characterized as fundamentally exceptional."

[46] In determining what constitutes exceptional circumstances, decisions of the Tribunal have found that the following factors may be considered (see for example *Decision No. 591/94*):

1. whether the employer had acted with due diligence in pursuing a SIEF claim after the employer knew, or ought to have known, of a “prolonged” recovery period or “enhanced” disability;
2. the nature of the disability - i.e. whether it involved a longer than usual recovery period; whether it involved a complex condition which made diagnosis and treatment difficult; or whether the pre-existing condition was unknown;
3. whether systemic delay resulted in a final decision outside the three [or four as appropriate]-year “window;” and
4. the elapsed time between the NEER cut-off date and the final SIEF decision because a longer elapsed time period could mean a more complex adjustment.

[47] In addition, the Board has drafted practice guidelines regarding exceptional circumstances for experience rating adjustments. The Board’s “Practice Guidelines: Experience Rating Adjustments – Exceptional Circumstances” is contained in the Board’s “Appeals Services Division Practice & Procedures.” The guidelines are not policy and the Tribunal is not required to apply them under section 126 of the WSIA. The Tribunal may, however, consider guidelines which are relevant and helpful to deciding the issues in dispute. The guidelines state that circumstances that may constitute exceptional circumstances under the merits and justice provisions of Board policy include, but are not limited to:

- Whether the employer pursued SIEF relief within a reasonable period after the employer knew or ought to have known the worker’s recovery period was prolonged or enhanced by a pre-existing condition.
- Whether there was a delay in identifying a pre-existing condition.
- Whether undue delay in the decision-making process caused the decision to grant SIEF relief to fall outside the experience rating window.
- The length of time between the closure of the experience rating window and the SIEF decision. It would be expected that discretion be extended in cases where the period is relatively short (i.e. less than six months).

...

[48] In this appeal, the accident year 2013 received its final NEER review on September 30, 2017. I find that the employer pursued SIEF within a reasonable period; however, I find that delays in the decision-making process caused the decision to grant an increase in the quantum of SIEF relief to fall outside of the experience rating window.

[49] The employer initially requested SIEF relief in a letter dated December 1, 2016, on the basis that the worker had two surgeries on his left knee and a third surgery was planned for December 2016 (and it was actually performed in January 2017). I note that as of December 1, 2016, the worker had only been in receipt of loss of earning (LOE) benefits for lost time as a result of medical appointments on May 13, July 7, July 8, July 21, 2016, and from July 23, 2016 (related to the revision surgery). I note that the worker had not received LOE benefits following his previous surgery on March 4, 2014 as he had not been employed at the time of his surgery, and as such, the Board concluded that he did not have a wage loss. I find that the delay by the employer in requesting SIEF relief until December 2016 was reasonable noting that there had not been any significant health care and loss of earnings benefits associated with this claim until 2016, which would have been likely to lead the employer to raise an issue related to a prolonged or enhanced disability in this claim. In this regard, I find that these circumstances

are somewhat similar to the circumstances in which there is a delay in identifying the pre-existing condition.

[50] The WSIB denied entitlement to SIEF in a decision dated January 23, 2017. The employer objected to this decision on February 2, 2017 and provided a submission dated January 30, 2017. The employer's representative provided another submission dated March 12, 2017. In a decision dated April 19, 2017, the CM granted SIEF at 50% in accordance with a moderate pre-existing condition and a moderate accident. The employer objected to the quantum of SIEF on April 19, 2017, which was upheld by the ARO in a decision dated September 20, 2017, which was prior to the final NEER review on September 30, 2017. The Notice of Appeal for Employers was faxed to the WSIAT on October 31, 2017.

[51] I find that due to the exceptional circumstances noted above, the employer is entitled to a retroactive adjustment of the employer's NEER account for the costs of this 2013 claim. In reaching this conclusion, I find that the employer acted with due diligence in pursuing an SIEF claim after the employer knew, or ought to have known, of a "prolonged" recovery period or "enhanced" disability. I also find that delays in the decision-making process caused the decision to grant an increase in SIEF relief to fall outside of the experience rating window.

[52] For the reasons stated above, I find that the employer is entitled to a retroactive NEER adjustment.

DISPOSITION

[53]

The appeal is allowed as follows:

1. The employer is entitled to an increase in SIEF cost relief which is currently at 50%, to 90%.
2. The employer is entitled to a retroactive NEER adjustment to reflect the 90% SIEF cost relief granted.

DATED: February 8, 2019

SIGNED: C. Huras